Health at the Curb: Autonomous Care for Aging America

LeConsultants







Overview | Executive Summary

Question

How can the U.S. Department of Health and Human Services (HHS) leverage technology to expand access to healthcare among underserved communities?



Goals

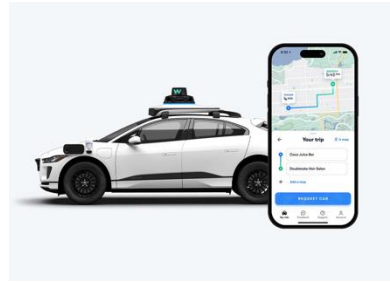
Address the **elderly community**	Integrate innovative **healthcare technology**	Expand reach through **scalable implementation**



Solution

WAY

Provide autonomous vehicle solutions allowing elders to perform routine monitoring and health check-ups at the convenience of their doorsteps

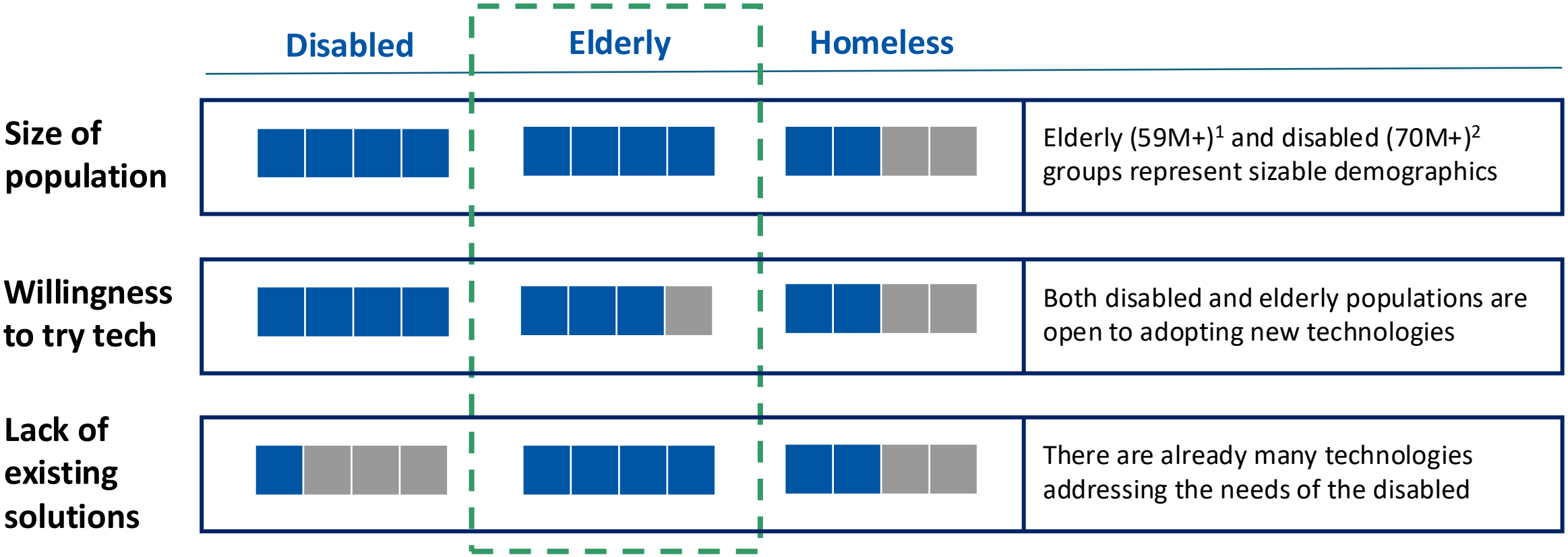


Impact

2.42x ROI By Year 5	**22%** 5-year Adoption Rate	**4M+** Users By Year 10



Overview | The elderly population represents a significantly underserved community



Among underserved groups, the elderly stand out for both the magnitude of their unmet needs and their readiness to benefit from impactful healthcare technology innovation at scale

Sources: [1]United Health Foundation, [2]CDC

Overview | The elderly population faces many pain points with current diagnostic processes



Elsa James

Living alone, son lives elsewhere

Limited mobility & transportation

Overdue for a health check-up

Committing To Diagnosis

Elder's avoidance of hospitals sees many early symptoms go unnoticed

Commuting To Appointment

Transportation barrier due to long commute and limited mobility

Scheduling Appointment

Limited availability of doctors could further deter action

Delayed/Rushed Appointments

Operational complications could negatively affect diagnosis quality

Overview | A reflection of the systematic issues within U.S. healthcare as a whole

The U.S. elderly population

1. 1 in 4 U.S. adults over the age of 65 live alone (16M+)[1]
2. Fear of burden leads to avoidance of health checks
3. 95% of elders have at least one chronic disease[2]

Key pain points within elderly healthcare

1. 21% of U.S. adults missed care due to no transportation[3]
2. Insufficient staffing leads to low provider-to-patient ratio
3. Poor experience discourages follow-up/routine visits

Key Takeaway

Systemic barriers – ranging from transportation and staffing shortages to emotional and experiential factors – prevent older adults from accessing routine care, highlighting the need for more equitable and accessible healthcare solutions

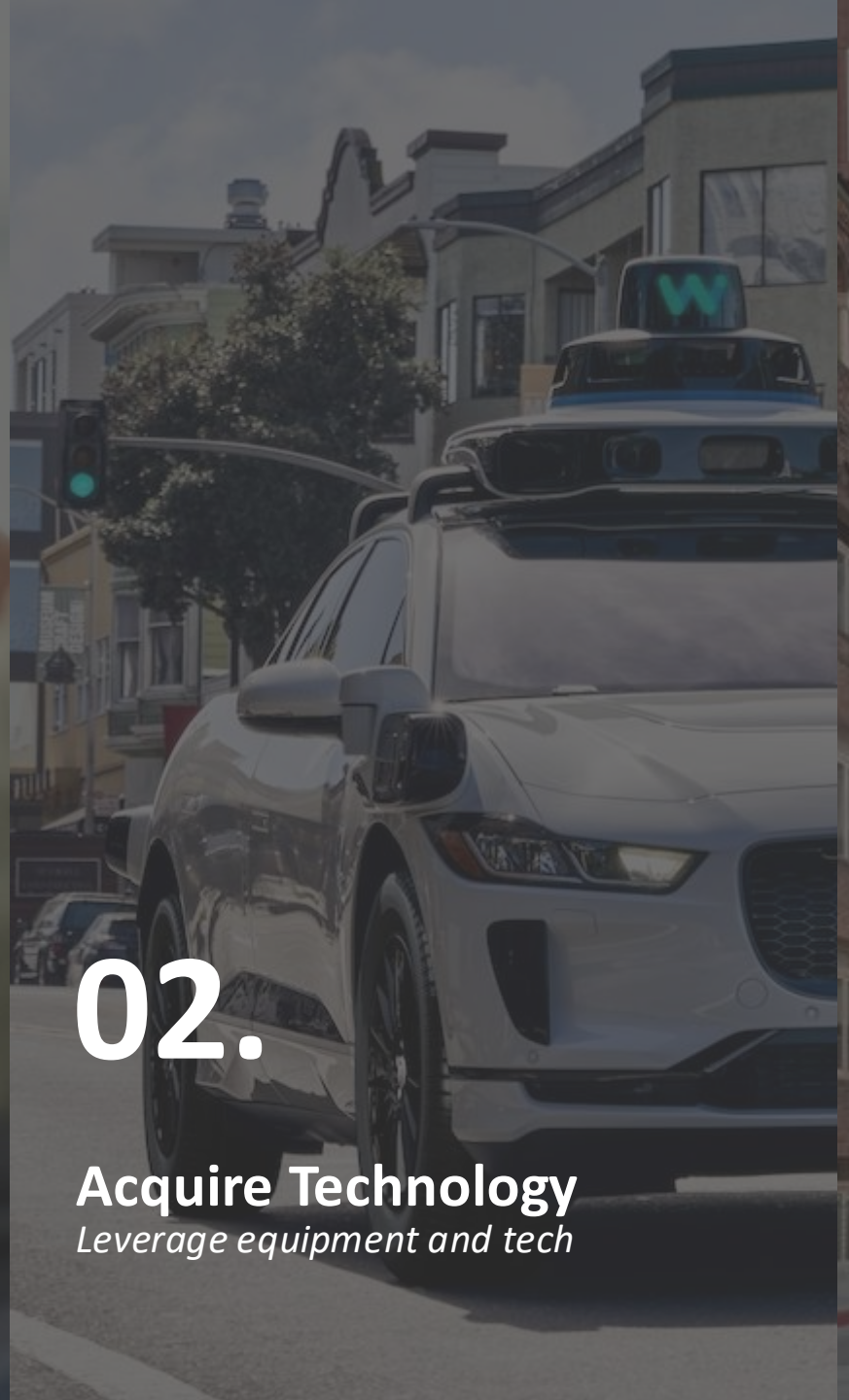
Sources: [1]Administration for Community Living, [2]NCOA, [3]Urban Institute



01.

Wellness on Demand

Accessing care without leaving home



02.

Acquire Technology

Leverage equipment and tech



03.

Yield Scale

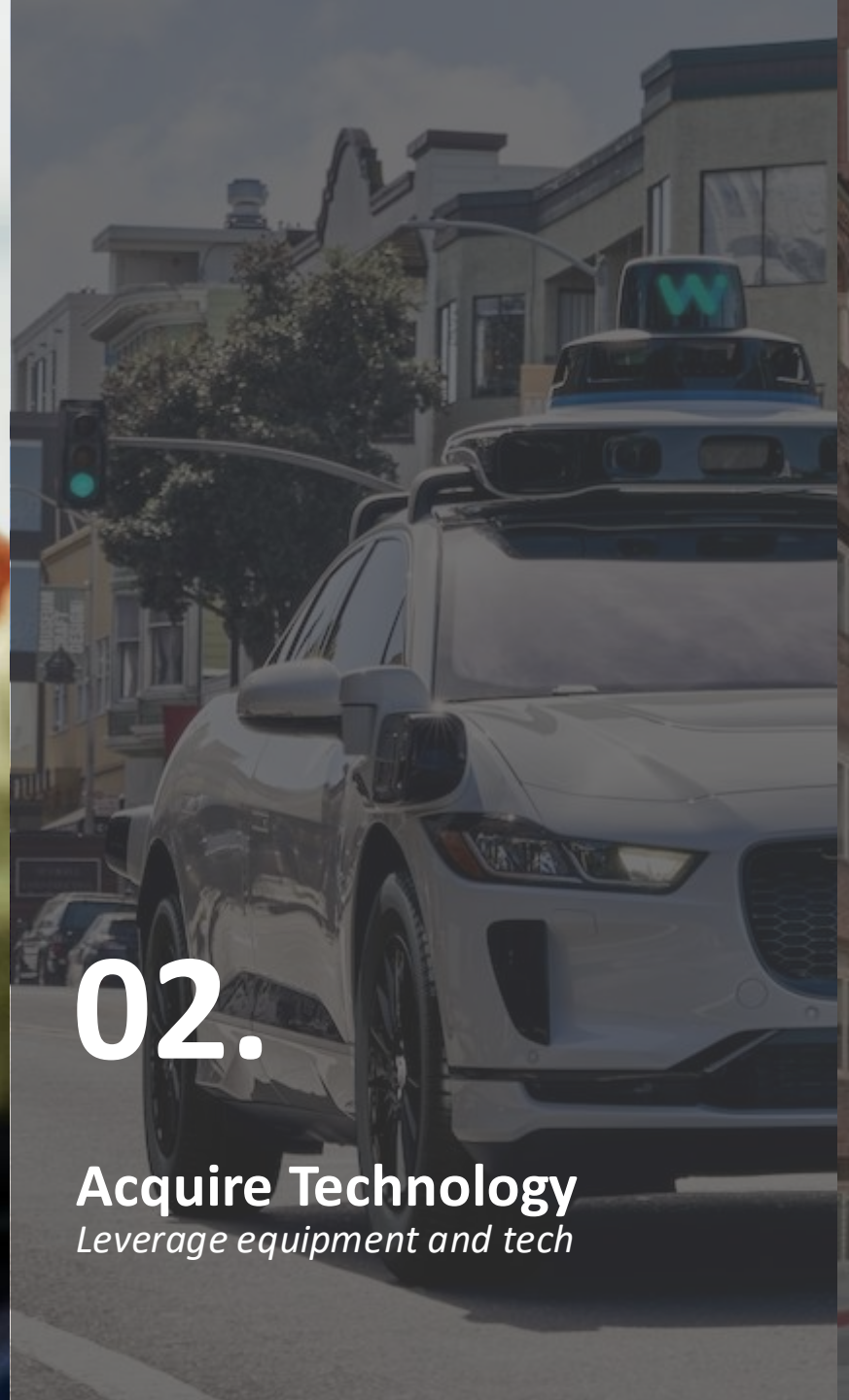
Pilot insights into scalable health equity



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Wellness | A multi-channel scheduling system will bring the health check-ups to elders, mitigating mobility barriers, unnecessary hospital visits and long patient wait times

User Flow

ACCESS

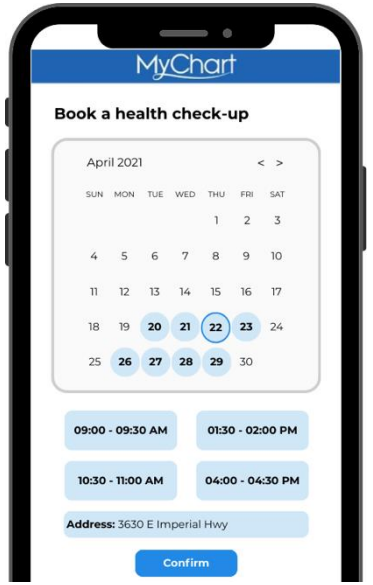
ASSESS

ADVANCE

Phone Call

1. Toll-free hotline doesn't require digital literacy
2. Offer interactive system and human support
3. Operator confirms name, address, preferred times
4. Booking confirmed with SMS and call reminder

Mobile App



Integrate scheduling system into existing patient portals

Community Centers

Elders won't engage if scared or unaware

Some can't navigate apps, calls, or screens

Meet them through familiar, trusted social touchpoints such as community and religious centers, and other NGOs like Meals on Wheels

Expands scheduling reach across diverse user types while making it user-friendly

Touchpoints for socially isolated elders during their rare in-person interactions

Source: [1]Pew Research Center (2019)

Wellness | HHS should deploy autonomous vehicles that enable self-guided health check-ups to increase care frequency, enhance patient independence, and integrate with existing health systems

User Flow

ACCESS

ASSESS

ADVANCE

Autonomous Check-Up

1. Verification

User scans their insurance card using a scanner before entering the van



Elders use **insurance ID** to access check-up system as they would for appointments



Secure **EHR connection** compares current results to past health data

2. Self Check-Up

User completes self-guided check-up; data syncs with EHR for health tracking









Assistance



Audio Assistance
Multi-language audio guides walk users through each step



Visual Assistance
Doctors and nurses demonstrate how to use the equipment



Live Assistance
Users can press a button to connect with a live assistant

This self-guided health check-up can help reduce the stress and fear of hospital visits and allow for more frequent and consistent data collection

Wellness | If further care is needed, users are automatically routed to the hospital with pre-shared data, reduced wait times, and transport support while easing pressure on clinical staff

User Flow

ACCESS

ASSESS

ADVANCE

1

Issue flagged during self-check-up

If vitals exceed safe thresholds based on the **user's past EHR data and the rule-based algorithm**, the system flags the issue

2

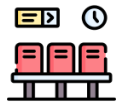
Hospital is notified and data is shared with them

Partner hospitals **receive the user's data** in real-time and the system **prioritizes cases based on severity and urgency** of observations

3

User is brought to the hospital using Waymo car

Users are **routed to the closest or their most-frequented hospital**, where a time slot is pre-booked to **reduce wait time**



Reduces patient wait time by notifying the hospital about upcoming visits



Frees up doctors and nurses by shifting basic screenings out of hospital



Improves access to hospitals by providing autonomous transport



Enables early diagnosis through frequent, low-barrier self check-ups



01.

Wellness on Demand

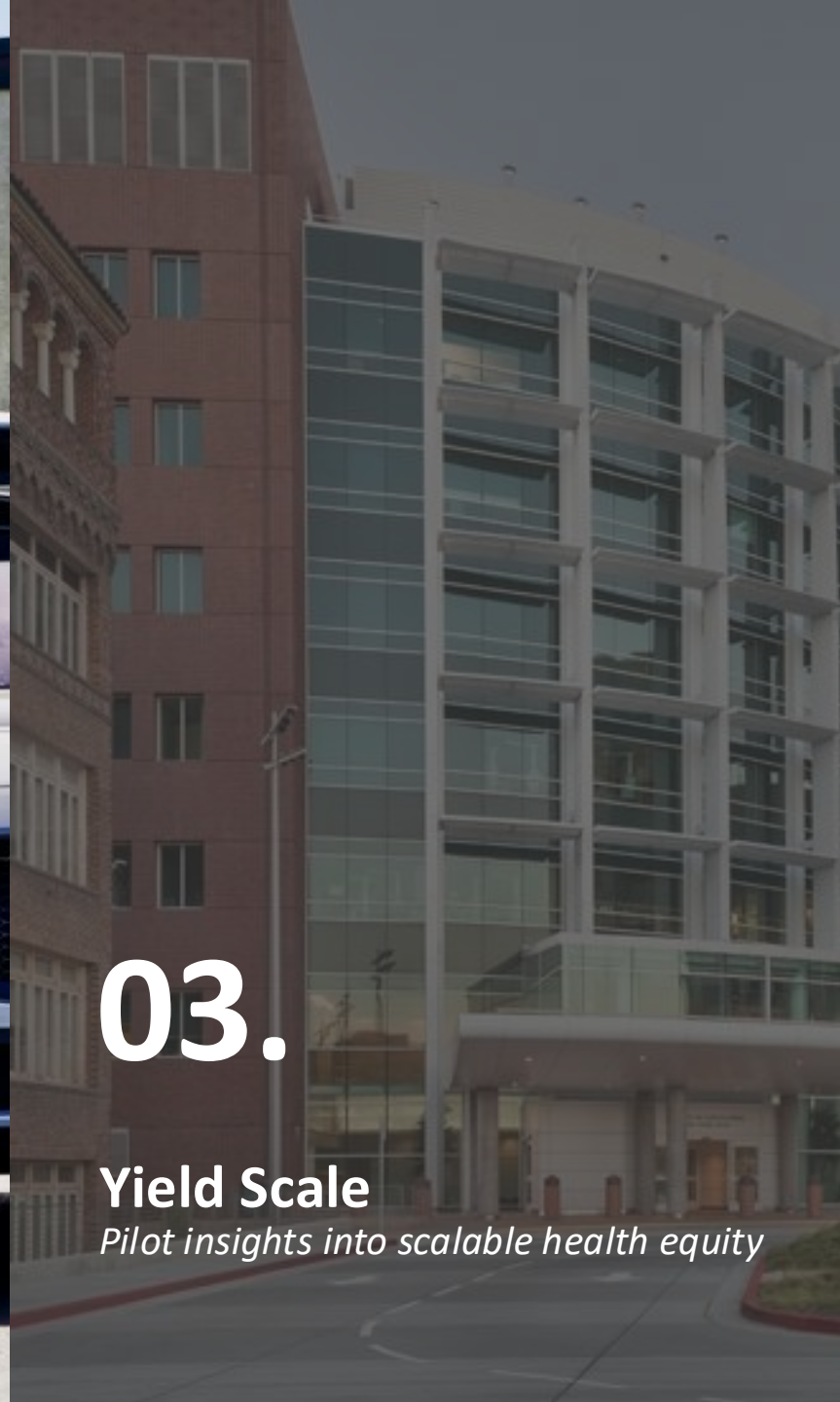
Accessing care without leaving home



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Pilot insights into scalable health equity

Acquire | By implementing devices specific to common elder conditions that are easy to use, we can receive key insights into early detection and research

Devices

Blood Pressure Monitor Cuff
Measures blood pressure





Bluetooth ECG Monitor
Measures electrical activity for the heart

Pulse Oximeter
Measures blood oxygen saturation levels





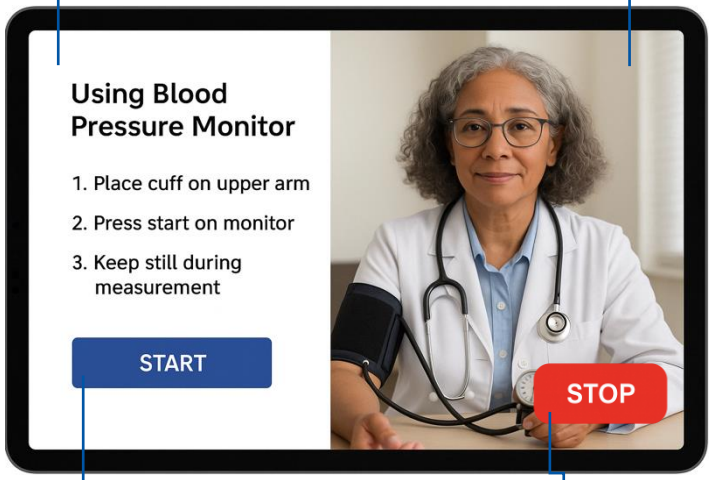
Glucose Meter
Measures blood sugar levels

Digital Stethoscope
Detects murmurs, fluid in lungs, and arrhythmias



Simple step-by-step instructions

Visual guidance



Intuitive start button to reduce confusion

Safety and precaution

Rationale

1 Common Elder Conditions

Allows us to screen for and monitor >90%[1] of diagnosed illnesses in elderly people

2 Easy Self-Use

All devices can be used while seated with minimal guidance and no needles

3 Actionable Data Insights

Create digitally created and clinically useful data used for early detection and research

Source: [1]Scientific Reports, Mayo Clinic, National Institute of Health

Acquire | Waymo offers plug-and-play infrastructure, government alignment, and a scalable path to healthcare equity—making them the ideal mobility partner for our solution

Strategic Advantages with Waymo

1. **Autonomous Accessibility**
   Shared mission of solving access barriers through technology



2. **Public Sector Alignment**
   Waymo's technology can realize improvements in elder care access



3. **Hardware-Software Integration Ready**
   Vehicle's software and connectivity is readily available for changes

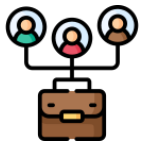


4. **Built in TV Screens**
   Leverage existing TV screens to use for visual instructions



Incentives for Waymo

Customer Acquisition



Establishes rapport with a new customer segment

Increased Exposure



Increased utilization rate and long-term growth in government partnership

ESG Offerings



Reducing healthcare inequities for seniors at scale

Waymo as a Partner

WAYMO

Leading the autonomous vehicle industry, we can partner with Waymo by **purchasing and retrofitting their vehicles** for mobile healthcare delivery

Acquire | Healthcare in Action's success proves that a multi-source funding model can sustain mobile health clinics with zero out-of-pocket cost

Healthcare IN ACTION
A MEMBER ORGANIZATION OF SCAN GROUP

Case Study: Funding and Pricing

HIS utilizes mobile health clinics to support unhoused individuals in communities across California

Primary Funding Sources

Cal-AIM and Medi-Cal
Received $3.8M[1] from California's CalAIM initiative



Health Insurers
Contact with local health insurers to provide services



Philanthropy
In 2022 a $500K[2] grant was donated to support initiatives



Pricing Strategies

Mirror an **ambulance pricing model** with a standardized base service fee. Like Healthcare in Action, offer a **zero out-of-pocket cost** by utilizing Medi-Cal reimbursements, health plan contracts, and government funding





Key Approach: Create a value-based, multi-source funding model that enables zero out-of-pocket cost to patients

Source: [1]HealthcareInAction [2]UniHealth

Acquire | Partnering with familiar community figures and communities leads to higher adoption rates and user confidence



Neighbors and Friends



Community Centers



Healthcare Professionals



Elsa James

I didn't sign up because of a website, I signed up because my neighbor told me it helped her

Source: [1]GrandPad

Gaining Credibility and Building Trust

1 Need	2 Approach	3 Impact
Less likely to trust unfamiliar services and technologies	Partner with local, senior trusted hubs to market service	Adoption is realized through community validation

Case Study: GrandPad

Partnered with organizations like local community centers to distribute the tablets and inform elders of its services

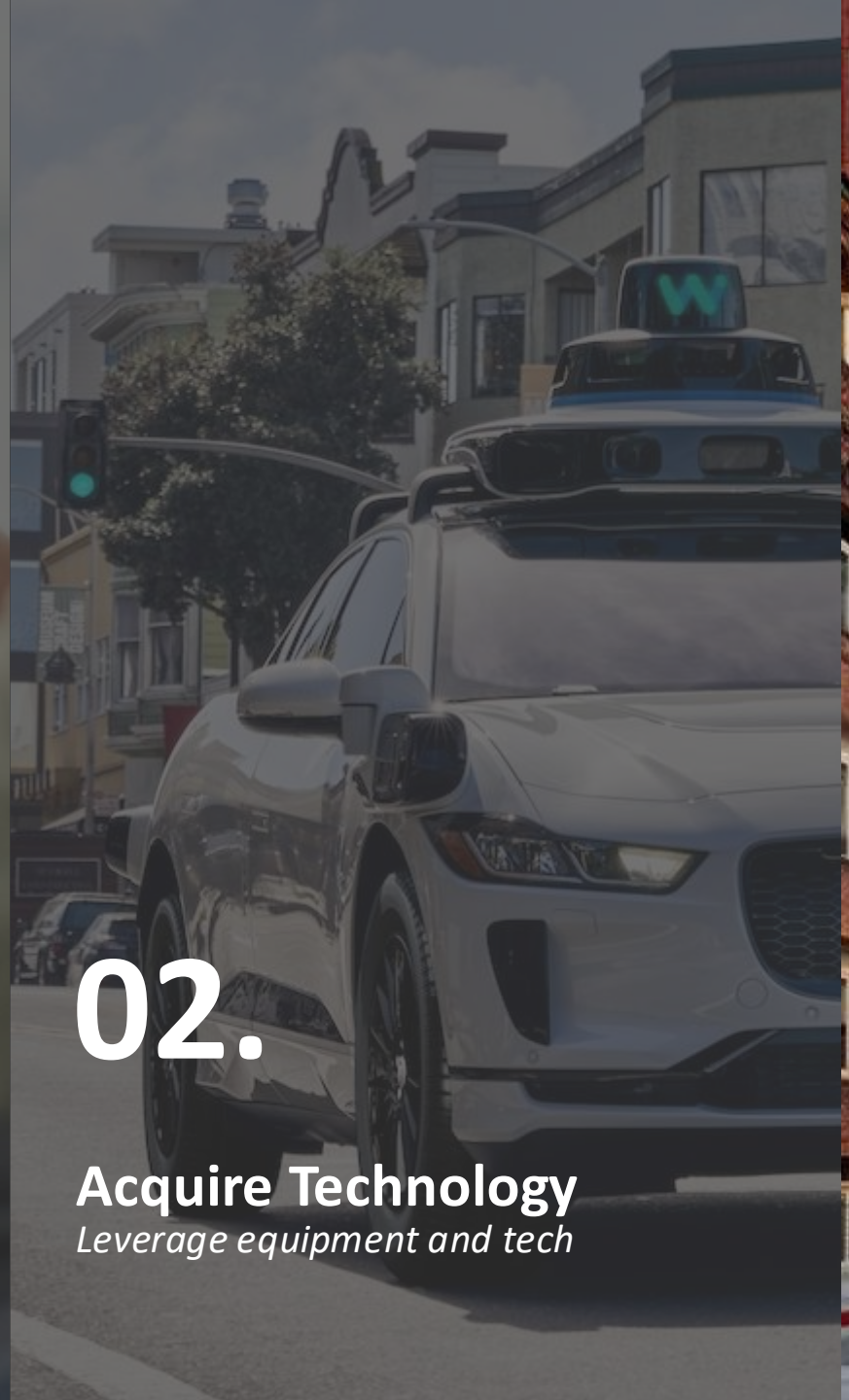
Led to higher engagement and user confidence in both elders and their kids[1]



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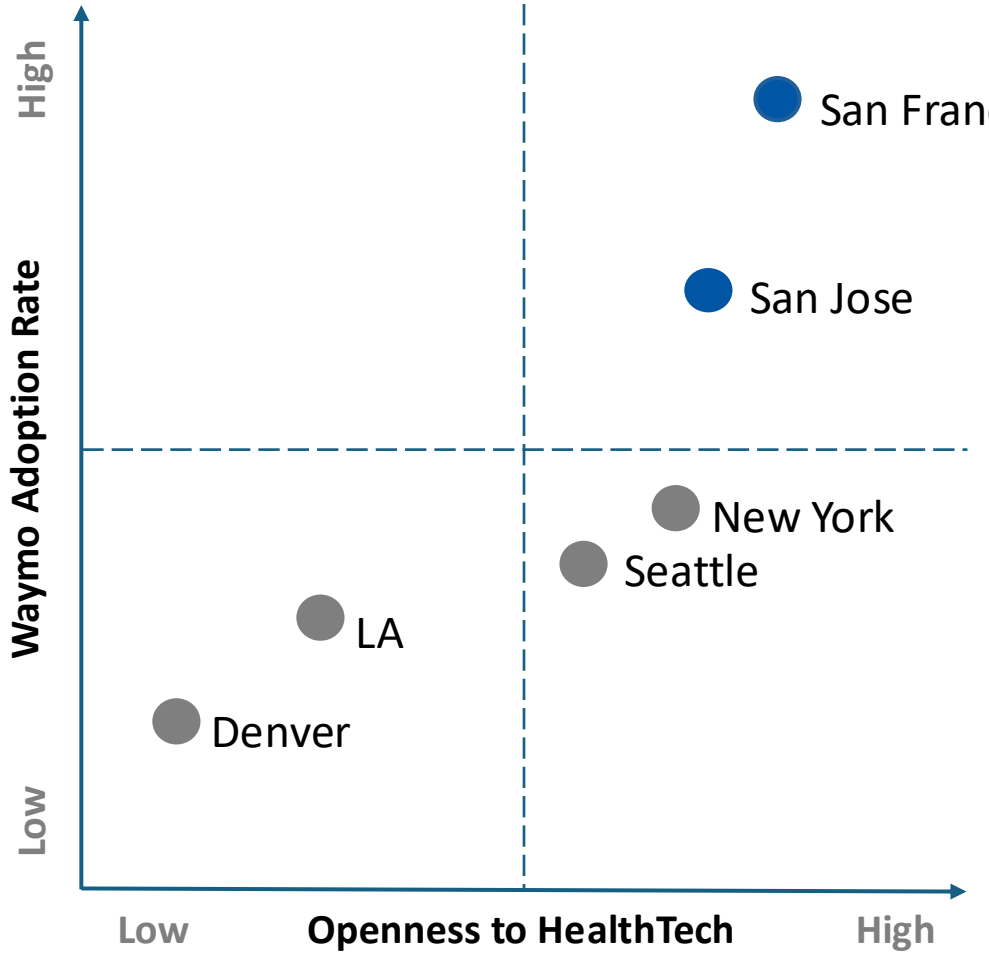


03.

Yield Scale

Pilot insights into scalable health equity

Yield | By identifying the optimal region to launch our service, HHS can maximize our social impact capabilities and set the stage for future expansion



Why Bay Area?

- High concentration of elderly individuals (~15%), especially in suburbs
- There are 17 potential public healthcare systems to partner with
- Tech literacy of senior citizens is higher than the national average

Why Suburban?

- Suburbs are less walkable and have limited transportation compared to downtown
- Suburban residents are likely long-term residents, prompting increased retention

Source: [1]Pew Research Center (2019)

Yield | A successful pilot program coupled with feedback mechanisms can foster long run geographic and industry expansion

Pilot Program



San Francisco will contain the first deployment of Waymo health check ups

Feedback



Encourage early adopters to provide feedback on pilot program

Expansion



Maximize scalability through geographic and industry expansion

Pilot Program Overview

- Based in Bay Area suburbs
- Target 5-year expansion in SF
- Attract users from hospitals and community
- Geographic expansion after 5+ years

Year	# of Hospitals	# of Waymos
Y1	2	4

Objectives

1. 17K+ total users in Y1
2. 50% conversion rate from hospitals
3. 13% conversion rate from community centers

Yield | A successful pilot program coupled with feedback mechanisms can foster geographic and industry expansion in the long run

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Digital Feedback



Following each consultation, elderly patients will be prompted to complete an online feedback survey

Feedback Goals

1. Reduced wait times
2. More convenient access
3. Easy to operate self diagnosis

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Maximize scalability through geographic and industry expansion

Geographic Expansion

1. **Y6:** Enter other city suburbs with Waymo coverage (e.g., Los Angeles suburbs, Phoenix suburbs)
2. **Y7-9:** Continue building trust and credibility in these areas (focus on increasing conversion rates YoY)
3. **Y10+:** Coincide with Waymo expansion and extend services to suburbs all around the nation

Industry Expansion



Routine prenatal check-ups for pregnant women in remote areas



Post-surgical follow-ups to monitor recovery process



Preventative care for specific chronic diseases (e.g., diabetes)

Impact | The implementation of mobile health clinics will solve current pain points for the elderly



Elsa James

- Living alone, son lives elsewhere
- Limited mobility & transportation
- Overdue for a health check-up

Routine scheduling of check-ups

Elder's will receive routine tests without the need detect symptoms themselves

Prioritize emergency scenarios

Elderly will only be transported to the hospital for emergencies, freeing up nurses and doctors

Convenient health monitoring

Services are brought to the elderly, taking away the painstaking commute to the hospital

Data collection and monetization

Elderly patient data will be routinely collected through EHRs and monetized for revenue

Yield | The implementation of mobile health clinics will solve current pain points for the elderly



Routine scheduling

What metrics can we use to track the success of this initiative?

& transportation



Overdue for a health check-up

Elderly will only be transported to the hospital for emergencies, freeing up nurses and doctors

Elderly patient data will be routinely collected through EHRs and monetized for revenue

Source: [1]Data

Impact | To track the success of the WAY strategy, numerous growth and healthcare impact KPIs should both be tracked

Growth

Adoption



22%

adoption rate within total serviceable obtainable market

Users



61K+

regular users in the Bay Area region within 5 years of launch

Healthcare Impact

Wait Time



8K+

hours saved annual on total wait times in public hospitals

Patients



11%

Increase in # of hospital patients served within the Bay Area

Partnerships

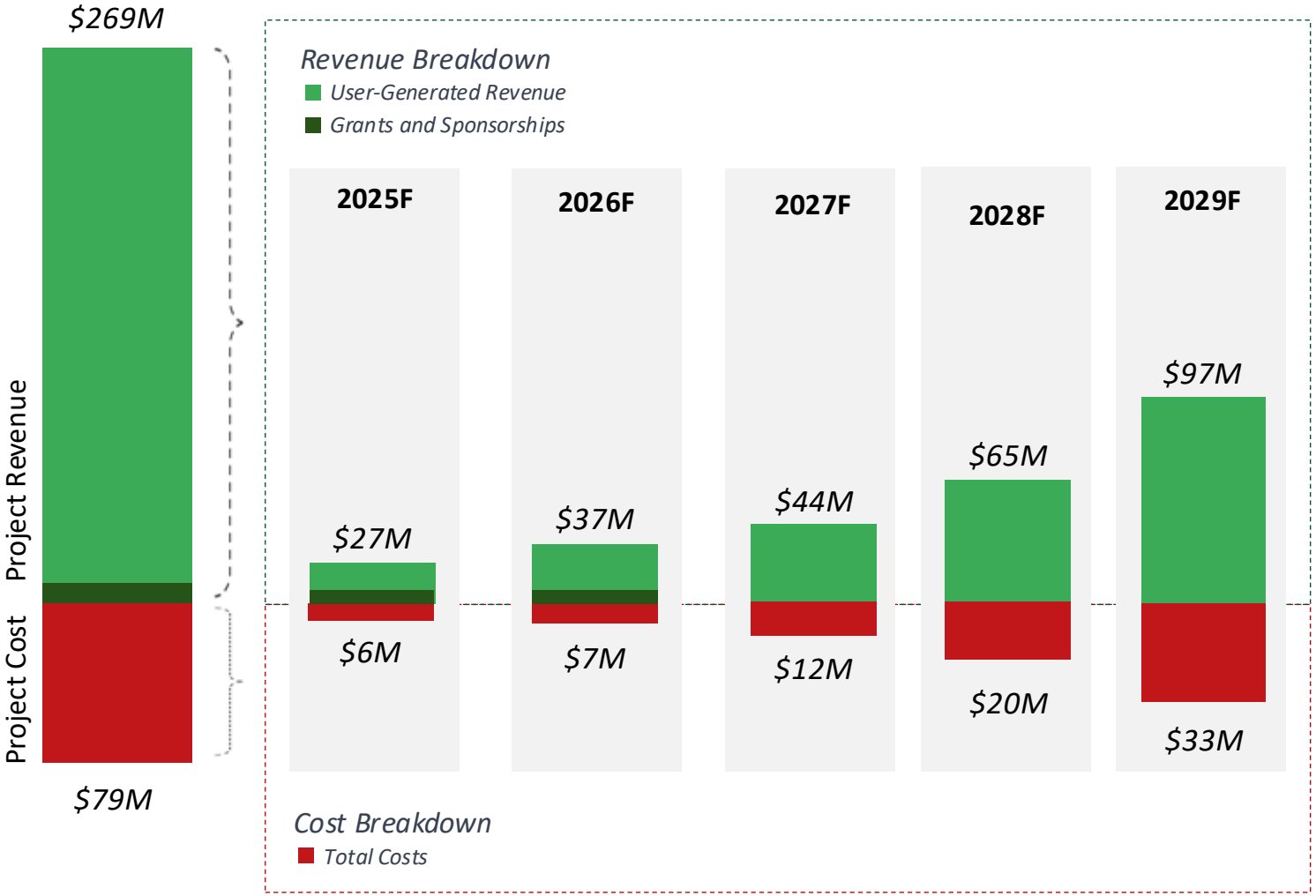


17+

hospital partnerships within the Bay Area Region

Impact | WAY will generate $269M in revenue and $79M in costs over the next fire years, resulting in a 2.42x ROI

Project Financials Over Time: *USD, Millions | Over 5 Years*



Key Revenue Drivers



$14M Grants & Sponsorships

40% Average User Growth Rate

18% YoY Conversion Rate Increase

3 Annual Uses per Customer

Key Cost Drivers



$3M Upfront Investment

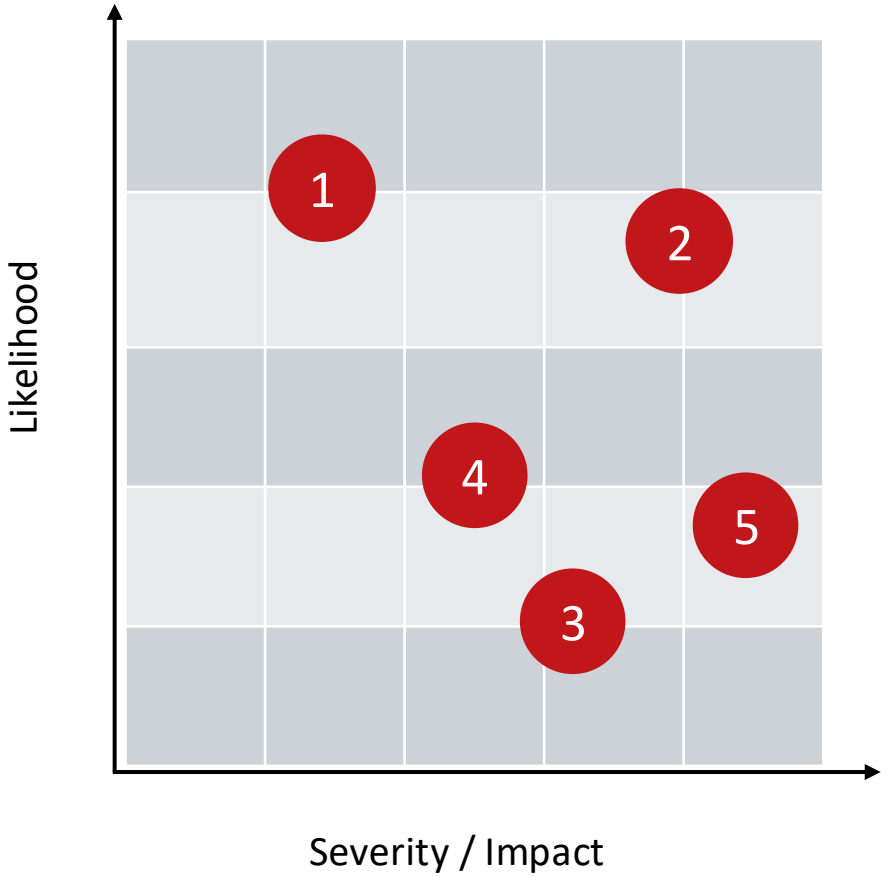
$13M Technology Implementation

60% Vehicle Costs

18% Total Partnership Costs

2.42x ROI over 5 Years

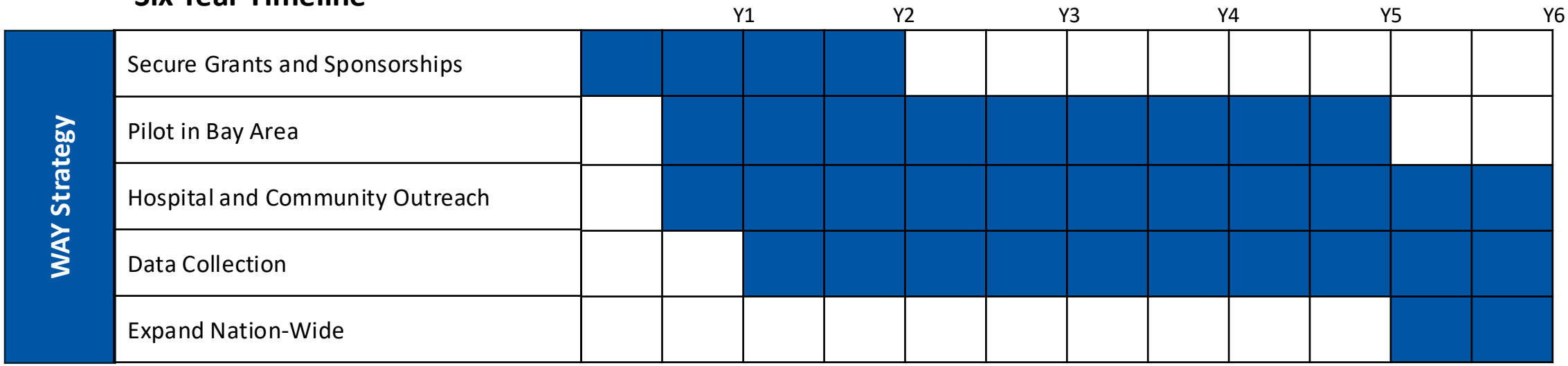
Impact | The WAY strategy may face the following risks and mitigations



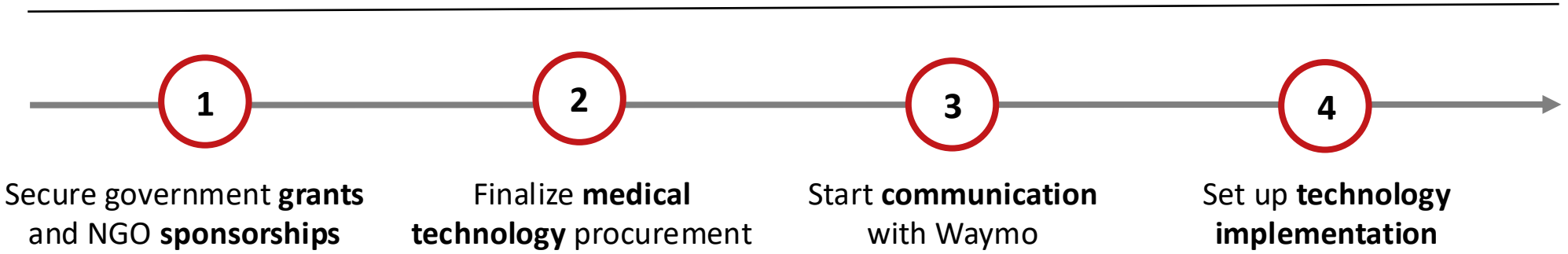
	Risks	Mitigation
1	Technical Issues with Vehicles	Maintain a tech support team and vet devices with hospitals
2	Low Adoption Among Elders	Introduce referral incentives within the elderly community
3	Data Privacy or EHR Sync Failures	Use HIPAA-compliant cloud storage and run regular cybersecurity audits
4	Delays in Grants and Sponsorships	Diversify funding with health plan contracts and local federal funds
5	Waymo Availability Constraints	Explore alternative AV platforms such as Cruise and Zoox

Impact | The WAY strategy will pilot and expand in the Bay Area for the first 5 years, then expand nation-wide

Six Year Timeline



Immediate Next Steps



Question

How can the U.S. Department of Health and Human Services (HHS) leverage technology to expand access to healthcare among underserved communities?



Goals

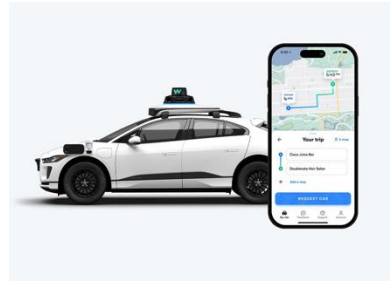
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Solution

WAY

Provide autonomous vehicle solutions allowing elders to perform routine monitoring and health check-ups at the convenience of their doorsteps



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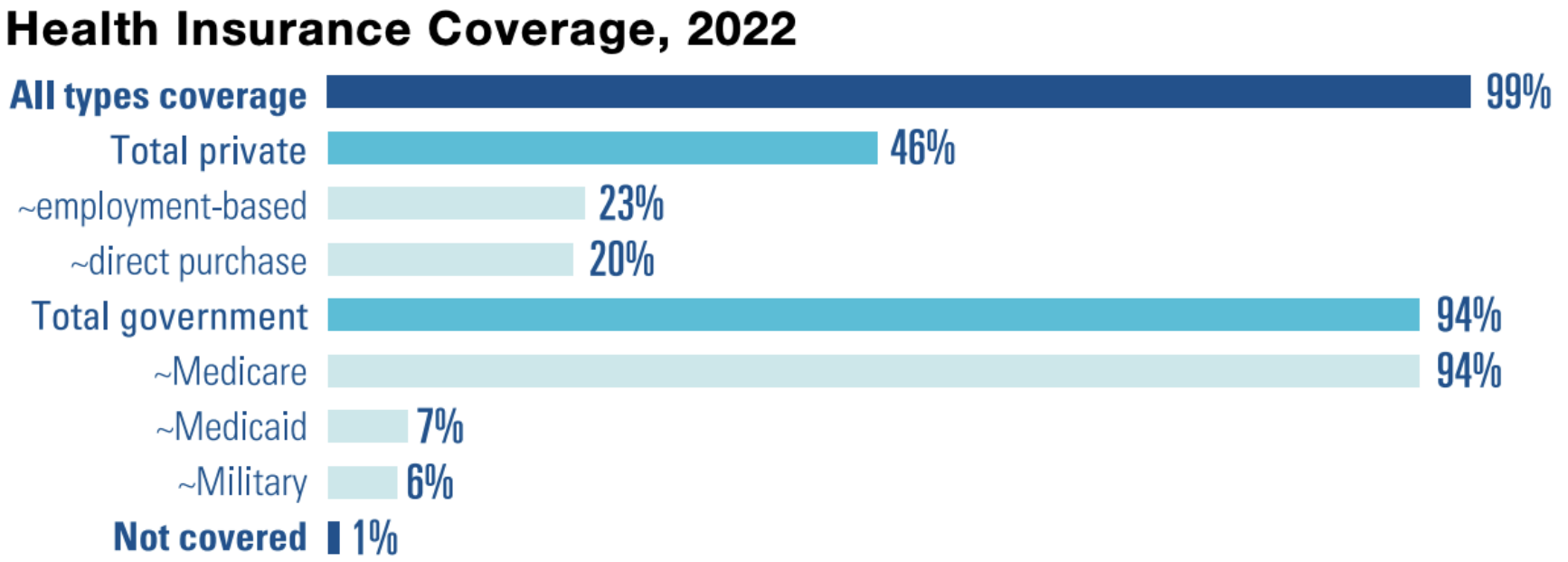
- **2.42x ROI** By Year 5
- **22%** 5-year Adoption Rate
- **4M+** Users By Year 10



Appendix



A1: Health Insurance
A2: Time Spent Alone + Factors
A3: Elders with Disabilities
A4: Elders Willingness to Use Technology
A5: Waymo Market Share Exceeds Lyft
A6: Justification for Devices
A7: Financial Support and Feasibility
A8: ROI Industry Standard
A9: Revenue Projections
A10: Cost Projections
A11: Total Usage Calculations
A12: Cost Breakdown
A13: Profit and ROI Calculation
A14: Time Saved By hospitals
A15: Bay Area TAM, SAM, SOM & User Adoption
A16: US TAM, SAM, SOM & User Adoption
A17: HIA Funding Structure
A18: Serviceable Obtainable Market Assumptions
A19: Wait Time Reduction Assumptions
A20: Other Strategies Considered
A21: Other Strategies Considered for Elders



Notes: 1) CPS/ASEC 2023 asks about insurance for 2022. 2) Military includes VA/ CHAMPVA/TRICARE

Source: U.S. Census Bureau, Current Population Survey, Annual Social and Economic Supplement

Source: [1]Administration of Community Living

Differences in time spent alone also emerge across educational levels. People ages 60 and older who have a high school diploma or less education spend, on average, 7 hours and 18 minutes a day alone – about 45 more minutes than their counterparts with a bachelor's degree. These patterns reflect in part the fact that less educated people are less likely to be married and living with a spouse than their more educated counterparts. (This is true among younger adults as well.) About half (51%) of adults ages 60 and older with a high school diploma are living with a spouse, compared with 59% of those with some college education or an associate degree and 67% of those with a bachelor's degree.

Living Arrangements

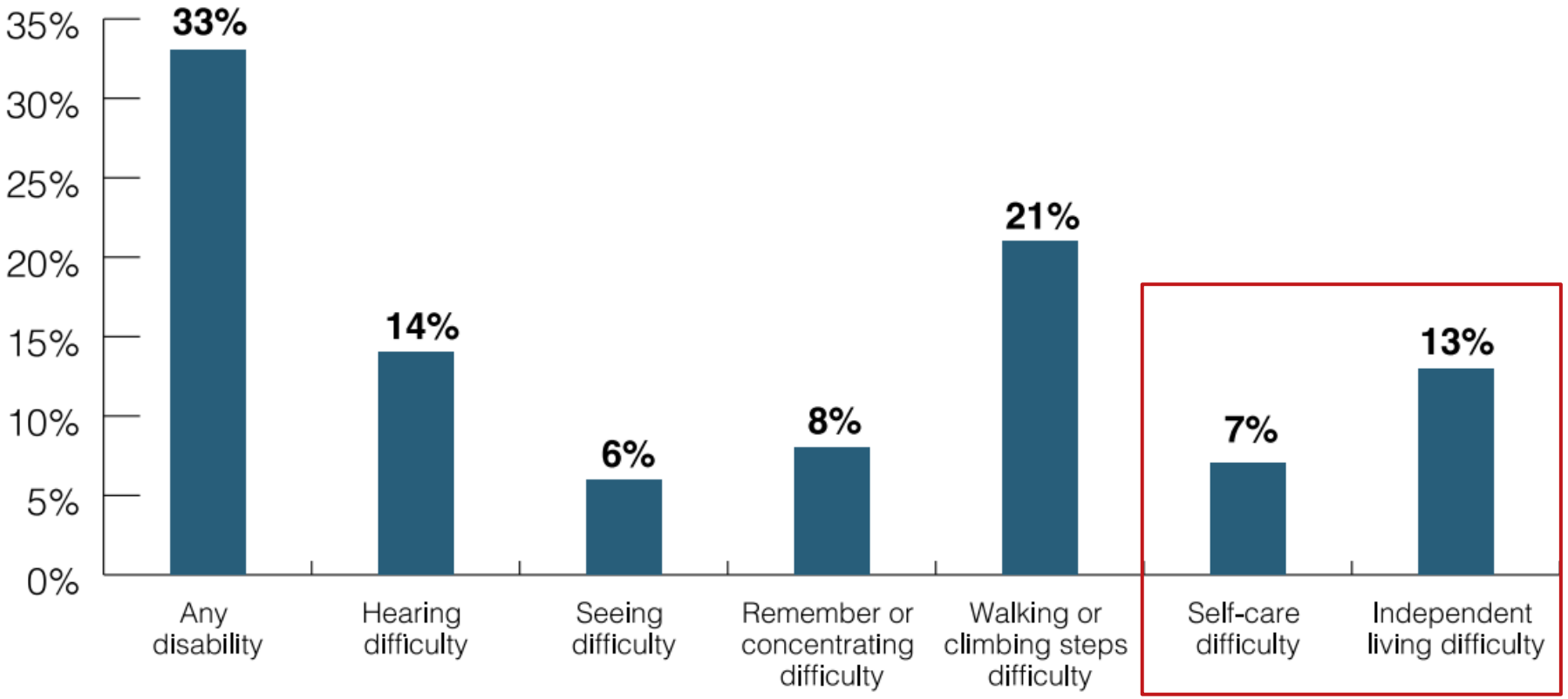
Of the older adults living in the community, more than half (59%) of people 65 and older lived with their spouse or partner in 2023. More than 18 million (71%) older men and 15 million (49%) older women lived with their spouse or partner.

In 2023, about 28% (16.2 million) of all older adults living in the community lived alone (5.7 million men, 10.5 million women). They represented 22% of older men and 33% of older women. The proportion of people living alone increases with advanced age for both men and women. Among women 75 and older, for example, 42% lived alone.

Educational attainment varied by age (see Figure 1.4). Across all racial and ethnic groups, adults ages 35 to 44 were the most likely to have attained an associate degree or higher (53.2 percent), followed by adults ages 25 to 34 (51.3 percent), adults ages 45 to 54 (50.8 percent), adults 55 to 64 (44.7 percent), and adults ages 65 and older (42.4 percent).

Source: [1]World Economic Forum, [2]American Council on Education, [3]Administration of Community Living

Percentage of Persons Age 65 and Older with a Disability, 2022



Source: [1]Administration of Community Living

Appendix | Elders' Willingness to Use Technology

Smartphone Ownership

As of 2025, 91% of adults aged 65+ own a smartphone, a dramatic increase from previous years. This widespread device ownership is the foundation for app usage[1]

App Usage

Over 50% of seniors use mobile apps regularly, especially for communication, entertainment, and health-related purposes[1]

Elders are very willing to try new technology…

In 2024, 67% of people aged 50+ made a tech purchase, and ownership of devices like smart home security systems and health-tracking apps has risen steadily[2]

…and of all types of tech, they

Many elders are motivated by technology that supports independence, safety, health, and social connection[3]

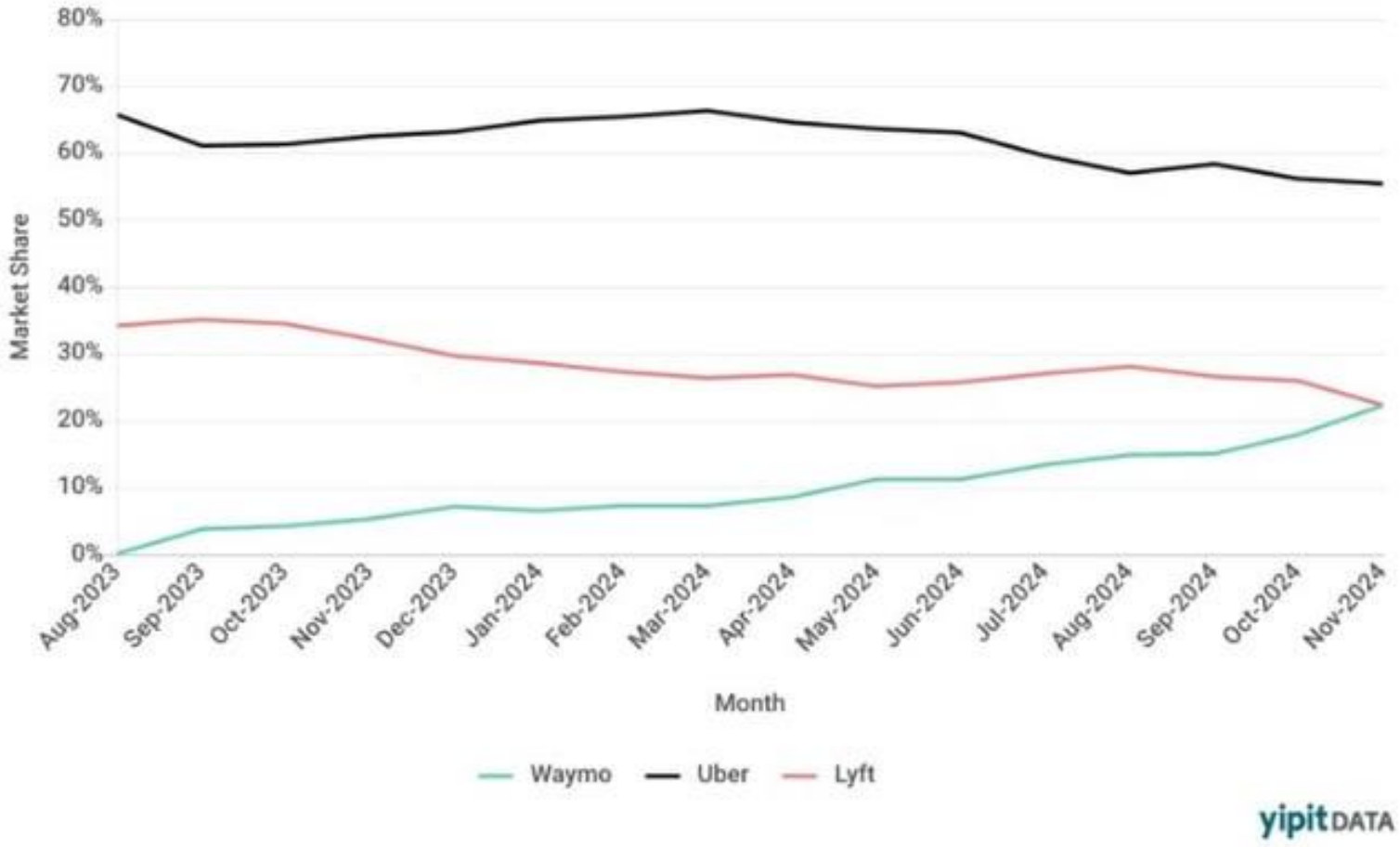
…but they demonstrate a "wait and see" approach….

While only about 11% of older adults consider themselves "early adopters," a much larger share (67%) are willing to try new technology once it is proven and established[2]

Source: [1]California Mobility, [2]American Association of Retired Persons, [3]Cyces

Appendix | Why Waymo

Exhibit 6. SF operating zone market share - gross bookings



Waymo's market share is now equal to Lyft within SF

Appendix | Justification for Devices Selected and Percentage of Diseases Covered

Hypertension is one of the major chronic diseases that leads to higher mortality worldwide. Early diagnosis and treatment of hypertension could significantly reduce the risk of mortality

The pooled sensitivity estimate was 79%, and the specificity was 91%. The meta-analysis of DOR suggests that the **digital blood pressure monitor** had moderate accuracy with a mercury sphygmomanometer. This will provide the clinician and patients with **accurate information** on blood pressure with which **diagnostic and treatment decisions** could be made

>70% of adults aged 65 and up have hypertension and **90%** of adults will develop it by age 80
>29% have diabetes and 40% are projected to develop diabetes in their lifetime
>14% have coronary heart disease
~10% of adults will develop COPD in their lifetime

An electrocardiogram (**ECG**) is a quick test to check the heartbeat. It records the electrical signals in the heart. Test results can help diagnose **heart attacks** and irregular heartbeats, called **arrhythmias**.

>94.9% of US adults aged 60 and older have at least one chronic disease and >78.7% have at least two diseases

Medical providers may use a pulse oximeter or may prescribe one for home use for patients with any of these conditions: Asthma, Lung cancer, **COPD**, **Influenza**, **COVID-19**, Sleep apnea, Pneumonia, Cystic fibrosis, **Heart disease**

Source: [1]Scientific Reports, [2]Mayo Clinic, [3]National Institute of Health, [4]National Council on Aging

Appendix | Financial Support and Feasibility

UniHealth Foundation, a non-profit philanthropic organization, and Cedars-Sinai awarded $500,000 in charitable funding to Healthcare in Action

SCAN Group is a mission-driven organization that is tackling some of the **biggest issues in health care for older adults**. SCAN Group operates SCAN Health Plan, one of the nation's foremost not-for-profit Medicare Advantage plans, serving more than 270,000 members in California.

Medi-Cal provides health care coverage to almost 40 percent of Californians

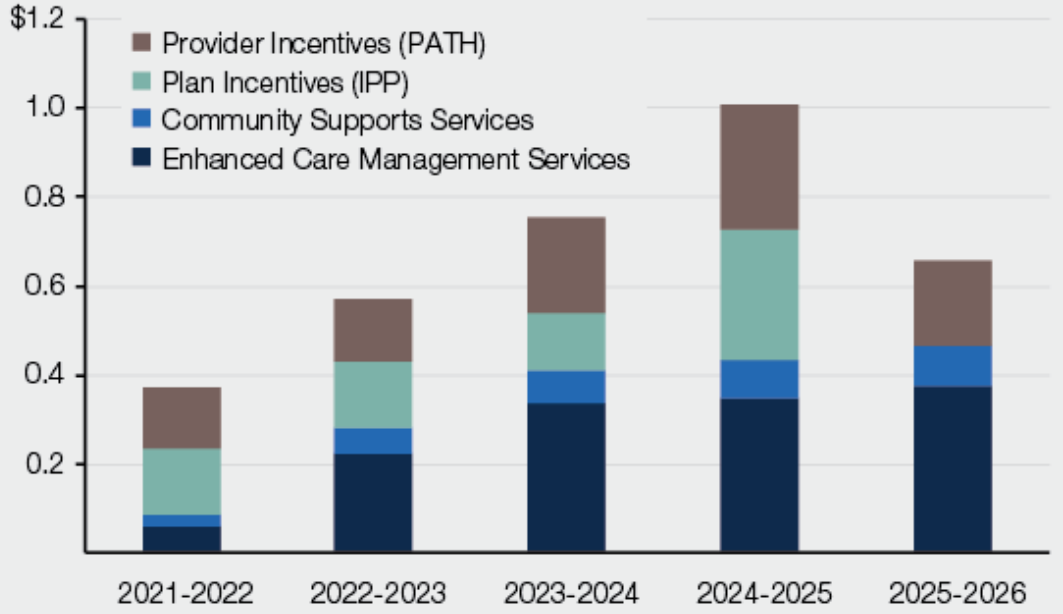
The Governor's 2025-26 budget proposes ECM spending at **$956 million (total funds)** and Community Supports spending at $231 million (total funds), a $67 million (7.5 percent) and $11 million (5 percent) increase over revised 2024-25 levels, respectively

(ECM) => Enhanced Care Management Program

Figure 4

CalAIM ECM, Community Supports, and Incentives Funding

General Fund (In Billions)



PATH and IPP funding reflect budgeted amounts, actual expenditures may differ.

CalAIM = California Advancing and Innovating Medi-Cal; ECM = Enhanced Care Management; PATH = Providing Access and Transforming Health Initiative; and IPP = Incentive Payment Program.

LAO

Source: [1]HealthcareInAction [2]UniHealth, [3]Legislative Analyst Office

Appendix | Industry Standard ROI

Return on investment of public health interventions: a systematic review

Rebecca Masters [1,2], Elspeth Anwar [2,3,4], Brendan Collins [2,4], Richard Cookson [5], Simon Capewell [2]

	Median ROI	ROI range	Number of ROI studies	Median CBR	CBR range	Number of CBR studies
Overall	**14.3**	–21.3 to 221	34	**8.3**	0.7 to 29.4	23
Local level	**4.1**	0.9 to 19.3	18	**10.3**	0.9 to 23.6	11
National level	**27.2**	–21.3 to 221	17	**17**	1.2 to 167.	10
Specialism						
Health protection	**34.2**	0.7 to 221	8	**41.8**	1.1 to 167	10
Legislation	**46.5**	38 to 55	2	**5.8**	3 to 8.6	2
Health promotion	**2.2**	0.7 to 6.2	12	**14.4**	2 to 29.4	3
Healthcare public health	**5.1**	1.1 to 19.3	6	None reported	None reported	None reported
Wider determinants	**5.6**	1.1 to 10.8	6	**7.1**	0.7 to 23.6	6

Public health interventions often prevent costly diseases or complications before they arise. They can help **avoid expensive treatments, hospitalizations, and long-term care**, leading to significant cost savings relative to initial investments.

Public health interventions also frequently include indirect benefits such as:

- Increased workforce productivity
- Monetized gains from improved quality of life
- Savings in social services (e.g., reduced crime, welfare dependency)

Appendix | Revenue Projections

Name	Unit	2025F	2026F	2027F	2028F	2029F
Grants and Donations						
Government Grants	$	4,800,000	4,800,000			
Charity Donations	$	2,270,000	2,270,000			
Total Public Revenue	**$**	**7,070,000**	**7,070,000**	-	-	-
			14,140,000			
User Payments						
Total # of Users	#	16,541	23,717	32,947	46,546	66,116
Price per Use	$	400	420	441	463	486
Average Usage/Year	#	3.01	3.01	3.01	3.01	3.01
Total Usage/Year	#	49,788	71,388	99,170	140,104	199,009
Total User Revenue	**$**	**19,915,385**	**29,983,036**	**43,734,026**	**64,875,189**	**96,758,435**
Total Revenue	**$**	**26,985,385**	**37,053,036**	**43,734,026**	**64,875,189**	**96,758,435**

Appendix | Cost Projections

Name	Unit	2025F	2026F	2027F	2028F	2029F
Vehicle Costs						
Vehicle	$	200,000	200,000	200,000	200,000	200,000
Maintenance	$	700	700	700	700	700
Medical Devices	$	9,000	9,000	9,000	9,000	9,000
Hardware Installation	$	1,900	1,900	1,900	1,900	1,900
Software Installation	$	2,500	2,500	2,500	2,500	2,500
On-Demand Voice/ Visual Assistance	$	5500	5500	5500	5500	5500
Live Assistance Implementation	$	4000	4000	4000	4000	4000
Total Cost/Vehicle	$	223,600	223,600	223,600	223,600	223,600
# of Vehicles	#	4	15	32	60	102
Total Vehicle Costs	**$**	**894,400**	**3,354,000**	**7,155,200**	**13,416,000**	**22,807,200**
Upfront Investment						
Diagnostic System Implementation	$	1,300,000				
EHR Integration	$	550,000				
Data Integration	$	660,000				
Licensing	$	570,000				
Total Upfront Investment	**$**	**3,080,000**	**0**	**0**	**0**	**0**
Technology Implementation						
Cloud Storage	$	378,000	945,000	1,512,000	2,268,000	3,213,000
Maintenance	$	30,000	33,000	36,300	39,930	43,923
Software Engineer	$	108,803	114,243	119,955	125,953	132,251
Data Scientist	$	113,301	118,966	124,914	131,160	137,718
IT Support	$	590,000	649,000	713,900	785,290	863,819
Total Tech Costs	**$**	**1,220,104**	**1,860,209**	**2,507,070**	**3,350,333**	**4,390,711**
Hospital Partnership Costs						
Hospital Partnerships	#	2	5	8	12	17
Website Remodelling	$	90,000	90,000	90,000	90,000	90,000
EHR Integration	$	85,000	85,000	85,000	85,000	85,000
Phone Call Integration	$	23,000	23,000	23,000	23,000	23,000
Partnership Cost/Hospital	$	198,000	198,000	198,000	198,000	198,000
Total Hospital Partnership Costs	**$**	**396,000**	**990,000**	**1,584,000**	**2,376,000**	**3,366,000**
Community Partnership Costs						
Outreach	$	25,500	25,500	25,500	25,500	25,500
# of Community Centers Reached	#	6	15	32	59	99
Total Community Partnership Costs	**$**	**153,000**	**382,500**	**816,000**	**1,504,500**	**2,524,500**
Miscellaneous Costs						
Legal Compliance	$	109,000	114,450	120,173	126,181	132,490
Total Miscellaneous Costs	**$**	**109,000**	**114,450**	**120,173**	**126,181**	**132,490**
Total Cost	**$**	**5,852,504**	**6,701,159**	**12,182,442**	**20,773,014**	**33,220,901**

Appendix | Total Usage Calculation

Name	Unit	2025F	2026F	2027F	2028F	2029F
Total Users	#	16,541	23,717	32,947	46,546	66,116
% Healthy	%	0.35	0.35	0.35	0.35	0.35
Usange if Healthy	peryear	1	1	1	1	1
% with Chronic Conditions	%	0.6	0.6	0.6	0.6	0.6
Usage if Chronic Conditions	peryear	3	3	3	3	3
% Unstable	%	0.03	0.03	0.03	0.03	0.03
Usage if Unstable	peryear	12	12	12	12	12
% Transitional	%	0.02	0.02	0.02	0.02	0.02
Usage if Transitional	peryear	25	25	25	25	25
Average Usage/Year	#	3.01	3.01	3.01	3.01	3.01
Total Usage/Year	#	49,788	71,388	99,170	140,104	199,009

Appendix | Cost Breakdown

Name	Unit	2025F	2026F	2027F	2028F	2029F	Total
Vehicle Costs		15%	50%	59%	65%	69%	60%
Upfront Investment		53%	0%	0%	0%	0%	4%
Technology Implementation		21%	28%	21%	16%	13%	17%
Hospital Partnership Costs		7%	15%	13%	11%	10%	11%
Commuinty Partnership		3%	6%	7%	7%	8%	7%
Miscellaneous Costs		2%	2%	1%	1%	0%	1%

Appendix | Profit & ROI Calculation

USD ($)	Unit	2025F	2026F	2027F	2028F	2029F	Sum
Total Revenue	$	26,985,385	37,053,036	43,734,026	64,875,189	96,758,435	269,406,072
Total Cost	$	5,852,504	6,701,159	12,182,442	20,773,014	33,220,901	78,730,021
Operating Profit	$	21,132,881	30,351,877	31,551,584	44,102,175	63,537,534	
Operating Margin	$	0.78	0.82	0.72	0.68	0.66	
ROI	X	2.42					

Appendix | Time Saved by Hospitals

Name	Unit	2025F	2026F	2027F	2028F	2029F
Hospital Partnerships	#	2	5	8	12	17
Average # of Patients/Hospital	#	5500	5500	5500	5500	5500
% of Elders as Patients	%	0.4	0.4	0.4	0.4	0.4
Average # of Elder Patients/Hospital	#	2200	2200	2200	2200	2200
Adoption Rate	%	0.06	0.09	0.12	0.17	0.23
Average Hospital Wait Time	Min	60	60	60	60	60
Time Saved/Hospital	Min	8,313	11,708	15,977	22,173	30,939
Time Saved/Hospital	Hour	139	195	266	370	516
Time Saved Total	Hour	277	976	2,130	4,435	8,766
Average Pre-Check Time/Patient	Min	45	45	45	45	45
New Patients Served/Hospital	#	185	260	355	493	688
Increase in % Served	%	0.03	0.05	0.06	0.09	0.13
New Patients Served Total	#	369	1,301	2,840	5,913	11,688

Appendix | Bay Area TAM, SAM, SOM & User & Adoption Rate Calculation

Name	Unit	2025F	2026F	2027F	2028F	2029F
Adoption Rate		0.06	0.09	0.12	0.17	0.23
Bay Area Population	#	7600000	7736800	7876062	8017832	8162152
Growth Rate	%	0.018	0.018	0.018	0.018	0.018
% of 65+ Elders	%	0.15	0.15	0.15	0.15	0.15
Total Bay Area Elders (65+)	#	1,140,000	1,160,520	1,181,409	1,202,675	1,224,323
% in Suburban Areas	%	0.60	0.60	0.60	0.60	0.60
Total Addressable Market	#	684,000	696,312	708,846	721,605	734,594
% with Barriers to Care	%	0.64	0.64	0.64	0.64	0.64
Servicable Addressable Market	#	437,760	445,640	453,661	461,827	470,140
% Willing to Try	%	0.6	0.6	0.6	0.6	0.6
Servicable Obtainable Market	#	262,656	267,384	272,197	277,096	282,084
Hospital Partnerships	#	2	5	8	12	17
Patients/Hospital	#	5500	5500	5500	5500	5500
% of Elders as Patients	%	0.4	0.4	0.4	0.4	0.4
% Converted	%	0.50	0.59	0.70	0.82	0.97
Total Users from Hospitals	#	2200	6490	12253	21688	36255
% of Elders Participating in Health Checkups	%	0.42	0.42	0.42	0.42	0.42
% Converted	%	0.13	0.15	0.18	0.21	0.25
Total Users from Community	#	14,341	17,227	20,694	24,858	29,861
Total Users	#	16,541	23,717	32,947	46,546	66,116
User Growth Rate	%		0.43	0.39	0.41	0.42
5-Year User Growth Rate	X					3
Adoption Rate Growth	%		0.41	0.36	0.39	0.40
5-Year Adoption Rate Growth	X					2.72

Appendix | US TAM, SAM, SOM & User Calculation

Name	Unit	2029F	2030F	2031F	2032F	2033F
Adoption Rate		0.06	0.09	0.12	0.17	0.23
US Population	#	346000000	349460000	352954600	356484146	360048987
Growth Rate	%	0.01	0.01	0.01	0.01	0.01
% of 65+ Elders	%	0.207	0.207	0.207	0.207	0.207
Total US Elders (65+)	#	71,622,000	72,338,220	73,061,602	73,792,218	74,530,140
% in Suburban Areas	%	0.60	0.60	0.60	0.60	0.60
Total Addressable Market	#	42,973,200	43,402,932	43,836,961	44,275,331	44,718,084
% with Barriers to Care	%	0.64	0.64	0.64	0.64	0.64
Servicable Addressable Market	#	27,502,848	27,777,876	28,055,655	28,336,212	28,619,574
% Using DHT	%	0.6	0.6	0.6	0.6	0.6
Servicable Obtainable Market	#	16,501,709	16,666,726	16,833,393	17,001,727	17,171,744
Total Users	#	1,039,211	1,478,342	2,037,526	2,855,924	4,024,771

Appendix | HIA Funding Structure

Healthcare in Action and SCAN's Medicare Advantage insurance plan generate revenue by serving homeless patients in multiple ways:

- Both are tapping into billions of dollars in Medicaid money that states and the federal government are spending to treat homeless people in the field and to provide new social services like housing and food assistance.

For instance, Healthcare in Action has received $3.8 million from Newsom's $12 billion Medicaid initiative called CalAIM, which allows it to hire social workers, doctors, and providers for street medicine teams, according to the state.

It also contracts with health insurers, including L.A. Care and Molina Healthcare in Southern California, to identify housing for homeless patients, negotiate with landlords and provide financial help such as covering security deposits.

- Healthcare in Action collects charitable donations from some hospitals and insurers, including CalOptima in Orange County and its own Medicare Advantage plan, SCAN Health Plan.
- Healthcare in Action partners with cities and hospitals to provide treatment and services. In 2022, it kicked off a contract with Cedars-Sinai to care for patients milling outside the hospital.
- It also enrolls eligible homeless patients into SCAN Health Plan because many low-income, older people qualify for both Medicaid and Medicare coverage. The plan had revenue of $4.9 billion in 2023, up from $3.5 billion in 2021.

Source: [1]ABC News

Appendix | Serviceable Obtainable Market Assumptions

In an online survey of more than 2,000 U.S. seniors ages 65 or older, **aging in place (69%), lack of transportation and access to medical care (64%),** and **economic insecurity (56%)** were the top three most-cited social barriers to their overall health.

According to a study published in JAMA Network Open, Most adults aged 50 to 80 in the United States are now using digital health technologies (DHTs). The analysis, based on data from over 2,100 participants and conducted by University of Michigan and Veteran Affairs Ann Arbor Healthcare System, found **81 percent** of older adults reported using at least one form of DHT, including telehealth services.

Source: [1]Alignment Health, [2]Healthcare IT News

- 68% of patients say the wait times at their practice are unreasonable.
- 30% of patients walked out due to an extended waiting room stay.
- 1 in 5 patients say they've switched doctors because of long wait times.

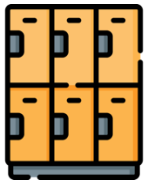
Average ER Wait Times by Severity and Location

ER wait times are affected by the severity of a patient's condition and the type of hospital facility. Here's an overview of what you might expect:

- **Severe, Life-Threatening Cases:** Immediate or near-immediate care; minimal wait.
- **Moderate, Urgent Conditions:** Wait times of 15–60 minutes, depending on triage protocols.
- **Non-Urgent Conditions:** Potential wait times of 1–4 hours, especially during peak periods.
- **Urban Hospitals:** Typically have longer wait times due to higher patient volumes.
- **Rural Hospitals:** Often have shorter wait times but may lack certain specialty services.

Source: [1]Solution Reach, [2]Frisco ER

Appendix | Other Strategies Considered

Alternate Strategy	Evaluation
 **Solar-Powered Smart Medicine Lockers** Regulated medicine lockers for low-income and homeless individuals without pharmacy access	 High risk of theft/fraud, regulatory concerns, uncontrollable environment factors
 **Ai-Powered Youth Mental Health** Partnering with universities and youth organizations to create AI diagnostic tools	 Privacy concerns and limited long-term use, unclear scalable impact
 **Health Wearables for the Disabled** Low-cost wearables to track post-treatment vitals and live condition updates	 Saturated market, unclear adoption incentives, low scalability

Source: [1]Solution Reach, [2]Frisco ER

Appendix | Other Strategies Considered for Elders

Alternate Strategy	Evaluation
 **Voice Assistant Health Monitor** Alexa-like device that checks vitals, notifying caregivers and family of complications	 Unclear education and distribution strategy, unclear scalable impact
 **On-Demand Home Health Services** Home installation of voice-activated health tech and remote patient monitoring tools	 High cost, distribution challenges, adoption barriers in rural regions, privacy concerns
 **Drones for Medical Supply Delivery** Using drones to move supplies from urban excess to rural shortages	 Pre-existing pharmaceutical delivery systems, weather and scalability limitations

Source: [1]Solution Reach, [2]Frisco ER